

2025 Professional Development Plan

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Reflection on Culturally Safe Practice

Current Foundation and Experience

My professional background includes over a decade of practice in underprivileged areas of Brazil, working extensively with large populations of Afro-descendants and Indigenous communities. This experience provided me with the clinical sensibility, awareness, and flexibility required to conduct patient and staff relationships with deep respect for diverse cultural symbols and traditions. I carry this foundational attitude of cultural humility into my current Australian practice, recognizing that clinical excellence is inseparable from cultural safety.

Improving and Applying Knowledge in the Australian Context

To specifically adapt my skills to the needs of Aboriginal and Torres Strait Islander peoples, I will implement the following actions within my scope of practice:

- **Environmental Safety:** I will ensure that our practice prominently displays the Aboriginal and Torres Strait Islander flags and includes a formal Acknowledgment of Country to create a welcoming and inclusive clinical environment.
- **Resource Integration:** I will use culturally appropriate educational materials and patient information to support health literacy through a culturally sensitive lens.
- **Clinical Application in Trauma:** In the context of the emergency care highlighted in my recent ATLS and PALS case reviews, I will apply a trauma-informed approach that recognizes the potential for intergenerational trauma. During acute presentations, I will prioritize effective communication with families and Elders to ensure that the patient's cultural needs are met alongside their clinical stabilization.

Reflection on Health Inequities

Current Foundation and Experience

My clinical experience in under-privileged suburban areas in Brazil has provided me with a deep understanding of how social determinants of health (SDOH), such as poor housing, lack of basic sanitation, food insecurity, and limited educational access, create systemic health inequities.

I observed how these factors not only increased the prevalence of chronic and infectious diseases but also served as significant barriers to treatment adherence and healthcare navigation. I bring this awareness to my practice in Australia.

Improving and Applying Knowledge in my Scope of Practice

To address health inequities within my current scope of practice, I will focus on the following strategies:

- Social Determinant Screening: I will integrate a broader "social history" into my standard consultations, specifically screening for barriers such as transportation limitations, financial strain related to medication costs, or unstable housing that may hinder a patient's recovery.
- Equitable Resource Allocation: I will ensure that I not only offer medication-based solutions but also advocate for access to multidisciplinary care (e.g., physiotherapy, psychology) for patients who might otherwise be priced out of these "beyond medication" management strategies.
- Trauma-Informed Advocacy: In acute trauma cases, such as those reviewed in my ATLS and PALS cases, I will reflect on the inequities inherent in recovery. I will work to ensure that post-discharge care plans are realistic and supported by community resources, recognizing that patients from lower socioeconomic backgrounds may lack the home-based support systems available to others.

Reflection on Professionalism and Ethical Practice

Current Foundation and Experience

Throughout my career, I have understood that professionalism is not merely about clinical expertise but also about the ethical responsibility to provide high-quality care under any circumstances. My recent work in Advanced Trauma Life Support (ATLS) and Pediatric Advanced Life Support (PALS) has reinforced that ethical practice requires a commitment to systematic discipline to avoid cognitive shortcuts that can lead to medical errors.

Improving and Applying Knowledge in my Scope of Practice

To uphold and enhance professionalism and ethical standards, I will focus on the following:

- Combatting Cognitive Bias: I will consciously apply the "systematic approach" learned in my trauma training to prevent "Distraction Bias". Ethically, I have a duty to treat every patient with the same level of investigative rigor, ensuring that obvious injuries do not distract me from life-threatening internal issues, such as the tension pneumothorax or airway obstruction identified in my recent case reviews.
- Transparency and Continuous Improvement: I will maintain a culture of transparency by conducting regular clinical audits of my own performance. As noted in my recent reflection on airway management, I identified a delay in accessing equipment; I will ethically address these system failures by advocating for better equipment layout in my clinical environment to protect future patients.
- Informed Consent and Communication: In both pediatric and adult trauma settings, I will ensure that my professional communication with families is honest and clear, even in high-pressure situations. This involves acknowledging when a clinical situation is critical while providing a structured plan of action to maintain trust and professional integrity.
- Peer Review and Accountability: I will continue to use tools like the RACGP CPD statement to track my progress and remain accountable to the Medical Board of Australia. This includes participating in performance reviews to ensure my clinical findings, such as those documented in my Physical Examination Workshop, remain aligned with current best practices.

Annual Reflection

This year, I have successfully fulfilled the core requirements of the RACGP CPD program, achieving a total of 64 CPD hours. This includes:

- Educational Activities (**EA**): 32.5 hours completed (Requirement: 12.5).
- Reviewing Performance (**RP**): 25.5 hours completed (Requirement: 5).
- Measuring Outcomes (**MO**): 6 hours completed (Requirement: 5).

A significant achievement this year was the successful completion of the ACS Advanced Trauma Life Support (**ATLS**) Course (22 hours) and the AHA Pediatric Advanced Life Support (**PALS**) Provider Course (16 hours). These courses, combined with my **Physical Examination Workshop** (19 hours), have substantially strengthened my clinical competencies in high-stakes emergency and pediatric care.

I have met my goal of applying a systematic approach to trauma, as evidenced by my two Clinical Case Reviews focused on airway management and pediatric pneumothorax.

For the 2026 cycle, my planning will focus on diversifying scope. Having focused heavily on emergency and trauma protocols this year, I intend to shift my focus toward chronic disease management and mental health to ensure a balanced scope of practice.